NEET-SS / FET / INI-SS HIGH-YIELD NOTES

Trauma

1. hip and femoral fractures
2. pelvis and acetabulum
3. tibial plateau and pilon
4. ankle fractures
5. proximal humerus and shoulder girdle

Hip and femoral fractures

1. Applied anatomy of the proximal femur

- **Neck-shaft angle** about 125-135 degrees; **anteversion** about 10-15 degrees in adults.
- **Blood supply of the femoral head:** mainly the **lateral epiphyseal (retinacular) branches of the medial femoral circumflex artery (MFCA)**, running up the posterosuperior neck. Lesser contributions: inferior gluteal, LFCA (anterior neck), and the artery of the ligamentum teres (small in adults; important in children). **Intracapsular fractures** disrupt the retinacular vessels: **AVN and non-union**.
- **Calcar femorale:** dense posteromedial cortical bone (load-bearing); **Ward's triangle** is the weak area between trabecular groups (Singh index grades osteoporosis from trabecular patterns).
- The **intertrochanteric line and capsule:** the capsule attaches anteriorly to the intertrochanteric line, but posteriorly only halfway down the neck.

2. Femoral neck fractures

Classifications

Garden	Description	Practical grouping
I	Incomplete / **valgus impacted**	Undisplaced
II	Complete, undisplaced	Undisplaced
III	Complete, **partially displaced** (trabeculae misaligned)	Displaced
IV	Complete, **fully displaced** (trabeculae of head realign with the acetabulum)	Displaced

- **Pauwels** (angle of the fracture line from horizontal; for young, high-energy fractures): **I under 30 degrees**, **II 30-50 degrees**, **III over 50 degrees (vertical shear, highest non-union and fixation failure)**.
- Location: subcapital, transcervical, basicervical (behaves like an extracapsular fracture).

Elderly (low-energy)

- **Undisplaced (Garden I-II): cannulated screws** (3 parallel screws in an inverted triangle, the inferior screw on the calcar, posterior screw against the posterior cortex; no screw entry below the lesser trochanter level: subtrochanteric fracture risk) or a sliding hip screw. Some evidence favours arthroplasty in very elderly frail patients with posterior tilt over about 20 degrees (high failure).
- **Displaced (Garden III-IV): arthroplasty**:
  - **Total hip arthroplasty** for active, independent, cognitively intact patients (better function; **HEALTH trial** showed small functional gains with more dislocations; NICE recommends THA for those who walk independently outdoors with a stick or less, are not cognitively impaired and are medically fit).
  - **Hemiarthroplasty** (bipolar or unipolar) for frailer, less active patients. **Cemented stems** are recommended (less pain, fewer periprosthetic fractures, better function: **WHiTE 5**), with care for **bone cement implantation syndrome**.
- **Orthogeriatric care:** surgery **within 36-48 hours** (NICE: on the day of or day after admission), early mobilisation, delirium prevention, nutrition, bone health (**secondary fracture prevention**: bisphosphonate, vitamin D), falls assessment. **One-year mortality about 25-30%.**

Young (high-energy)

- An **emergency**: **urgent anatomical reduction** (closed or open, Watson-Jones or Smith-Petersen approach) and **stable fixation** (cannulated screws, sliding hip screw ± derotation screw, or a fixed-

angle device such as a **dynamic hip screw with anti-rotation screw** or **proximal femoral locking/medial buttress plate** for vertical Pauwels III fractures).

- Complications: **AVN (about 20-30%)**, **non-union (about 10-30%)** (valgus intertrochanteric osteotomy (**Pauwels osteotomy**) converts shear into compression for non-union in young patients).
- **Ipsilateral femoral neck and shaft fracture:** the neck fracture (vertical, basicervical, undisplaced) is **missed in about 20-30%**: get a dedicated CT/fine-cut through the neck. Prioritise the **neck** (anatomical reduction).

Stress fractures of the femoral neck (Fullerton/Devas)

- **Compression (inferomedial)**: protected weight bearing; fix if the line exceeds about 50% of the neck or does not heal.
- **Tension (superolateral)**: high risk of displacement: **fix with screws**. Female athlete triad / RED-S.

3. Intertrochanteric (pertrochanteric) fractures

- Extracapsular; good blood supply, **unions reliably**; problem is fixation failure (**cut-out**).
- **Classification (AO/OTA 31A):**
  - **A1:** simple pertrochanteric (two-part; stable; intact lateral wall).
  - **A2:** multifragmentary pertrochanteric (posteromedial comminution, lateral wall incompetent in A2.2-2.3 per 2018).
  - **A3: intertrochanteric/reverse oblique** (unstable).
  - Evans (stable vs unstable; reverse obliquity is unstable), Boyd and Griffin, Jensen.
- **Unstable features: posteromedial comminution** (lesser trochanter), **reverse obliquity**, **subtrochanteric extension**, **lateral wall fracture** (thickness under about 20.5 mm predicts failure; Hsu), and inability to reduce.
- **Implants:**

  - **Sliding (dynamic) hip screw (DHS)** for **stable** fractures (A1): controlled collapse along the barrel.
  - **Cephalomedullary nail** (PFN, PFNA, Gamma, TFN, InterTAN) for **unstable** fractures, **reverse oblique** and subtrochanteric extension, deficient lateral wall. **DHS is contraindicated in reverse obliquity** (the fracture slides medially along the lag screw axis).
- **Tip-apex distance (TAD, Baumgaertner):** sum of the distances from the lag screw tip to the head apex on AP and lateral views, corrected for magnification. **Keep under 25 mm** (cut-out risk rises sharply above 25 mm). Position **centre-centre or inferior-centre** on AP, centre on lateral.
- **Complications: cut-out** (commonest mechanical failure; poor TAD, superior/anterior screw placement, unstable pattern), **Z-effect** (two-screw nails), **varus collapse**, periprosthetic fracture at the tip of a short nail, non-union (rare), malrotation.

4. Subtrochanteric fractures

- Within **5 cm distal to the lesser trochanter**. **High stress concentration** (medial compression, lateral tension).
- Deforming forces on the proximal fragment: **flexion (iliopsoas), abduction (gluteus medius/minimus) and external rotation (short external rotators)**; the shaft is pulled into **adduction (adductors)** and shortening.
- **Russell-Taylor** (piriformis fossa and lesser trochanter involvement); Seinsheimer.
- **Treatment: cephalomedullary (reconstruction) nail**, often after **open or clamp-assisted reduction** (reduction is the key; a malreduced varus fracture fails). Plates (95-degree blade plate, locking plates) in selected cases.
- **Atypical femoral fractures (AFF) with long-term bisphosphonates (ASBMR criteria):** subtrochanteric or diaphyseal, **minimal or no**

trauma, **transverse or short oblique** with a **medial spike** when complete, **non-comminuted**, **lateral cortical "beaking"/thickening**, prodromal thigh pain, often **bilateral** (image the other femur). **Stop the bisphosphonate**, calcium/vitamin D, consider **teriparatide**; **prophylactic intramedullary nailing** for incomplete fractures with pain or a visible fracture line.

5. Femoral shaft fractures

- High-energy injuries; blood loss up to about 1-1.5 litres; **fat embolism** risk; associated injuries (ipsilateral neck fracture, knee ligament injuries, hip dislocation).
- **Winquist-Hansen** comminution grades 0-IV.
- **Treatment of choice: reamed, statically locked antegrade intramedullary nail** within 24 hours in a stable patient. **Early fixation reduces pulmonary complications** (ARDS, fat embolism) and allows mobilisation.
- **Antegrade nail entry: piriformis fossa** (straight nails) or **trochanteric** entry (curved-proximal nails; avoids iatrogenic neck damage; small abductor damage). **Retrograde nail** (through the intercondylar notch): obesity, bilateral fractures, **ipsilateral femoral neck fracture** (with separate neck fixation), floating knee, pregnancy, distal fractures, ipsilateral acetabular fracture.
- **Reaming** increases union rates (**SPRINT for tibia** showed no harm; for femur, reamed nails union better); theoretical fat embolism risk.
- **Damage control orthopaedics (DCO): external fixation first**, nail later (within about 2 weeks; ideally day 4-14 when the inflammatory "second hit" settles), in **borderline or unstable** polytrauma patients (see polytrauma notes).
- **Complications:**
  - **Malrotation** (commonest malalignment after nailing; compare with the other side; over about 15 degrees is significant).

- **Limb-length discrepancy**, **non-union** (about 1-5%; exchange nailing for aseptic hypertrophic non-union), malunion (valgus with proximal fractures, apex-anterior with distal entry).
- **Fat embolism syndrome:** 24-72 hours, **Gurd and Wilson criteria** (major: respiratory insufficiency, cerebral signs, **petechial rash** (axilla, conjunctiva)); supportive care.
- Pudendal nerve palsy (traction table perineal post), heterotopic ossification at the entry, knee pain (retrograde).

6. Distal femur fractures

- **AO/OTA 33A (extra-articular), B (partial articular; B3 = coronal "Hoffa" fracture), C (complete articular).**
- **Hoffa fracture:** coronal plane fracture of a femoral condyle (**lateral more common**); seen only on lateral views/CT; fix with **anteroposterior lag screws** (perpendicular to the fracture).
- **Treatment: lateral locking plate** (MIPO) or **retrograde nail** (extra-articular and simple articular); **dual plating (lateral + medial)** for comminuted, bone loss or osteoporotic fractures and in nonunions; **distal femoral replacement** in elderly osteoporotic patients with very distal or periprosthetic fractures.
- **Deforming forces:** the **gastrocnemius** pulls the distal fragment into **extension (apex posterior)**; quadriceps and hamstrings shorten.
- **Periprosthetic distal femur fracture above a TKA:** Lewis-Rorabeck (I undisplaced, II displaced with a stable prosthesis, III loose prosthesis) or Su classification; fix (plate, retrograde nail if the box allows) or revise/replace if loose.

7. Patella (often with knee trauma)

- Undisplaced with intact extensor mechanism (can straight-leg raise): cylinder cast/brace.

- **Displaced (step-off over 2 mm or gap over 3 mm) or extensor lag: tension-band wiring** (K-wires + figure-of-eight; converts tension to compression) or cannulated screws with tension band; partial patellectomy for comminuted inferior pole.
- **Bipartite patella:** superolateral (Saupe type III), bilateral, smooth rounded margins: not a fracture.

Exam pearl: **Garden III-IV in the elderly = arthroplasty (cemented; THA if active and cognitively intact). Young displaced neck fracture = emergency anatomical reduction. Pauwels III = vertical shear, highest failure.**

Exam pearl: **TAD under 25 mm prevents cut-out. DHS for stable; nail for unstable, reverse oblique and subtrochanteric. Atypical femoral fracture: lateral cortical beaking, transverse, bilateral: stop bisphosphonates, consider teriparatide, nail.**

Exam pearl: **Ipsilateral neck + shaft fracture: the neck is often missed; prioritise the neck.**

Pelvis and acetabulum

1. Applied anatomy

- The pelvic ring is stable because of its **posterior ligamentous complex**: the **posterior sacroiliac ligaments (strongest ligaments in the body)**, interosseous SI ligaments, sacrospinous (resists external rotation) and sacrotuberous (resists vertical shear and flexion) ligaments, and the iliolumbar ligaments. **Posterior stability matters more than anterior.**
- Anterior: pubic symphysis (fibrocartilage); normal symphyseal width under about 5 mm, SI joint under about 4 mm.
- **Bleeding:** about 80-90% is **venous** (presacral and prevesical plexus) or from cancellous bone; about 10-15% is **arterial**, most commonly the **superior gluteal artery** (posterior, at the greater sciatic notch), then internal pudendal and obturator. The **corona mortis** (anastomosis between external iliac/inferior epigastric and obturator vessels, behind the superior pubic ramus, about 4-6 cm from the symphysis) is at risk in the Stoppa and ilioinguinal approaches.
- **Nerves:** the **L5 root** crosses the sacral ala (at risk with iliosacral screws placed too anteriorly); sacral roots in zones II/III sacral fractures; the lumbosacral trunk.

2. Pelvic ring injuries

Radiographs and CT

- AP pelvis, **inlet view** (beam angled caudally about 25-45 degrees: shows **AP displacement and rotation**) and **outlet view** (beam angled cephalad about 40-45 degrees: shows **vertical displacement** and sacral foramina). **CT** is standard.
- **L5 transverse process avulsion** suggests vertical instability. **Ischial spine avulsion** suggests sacrospinous disruption.

Young-Burgess (mechanism-based; predicts resuscitation needs)

Type	Mechanism	Injury pattern	Notes
APC I	Anteroposterior compression	Symphysis widening **under 2.5 cm**, SI ligaments intact	Stable; non-operative
APC II	AP compression	Symphysis **over 2.5 cm**, **anterior SI ligaments torn**, posterior SI intact ("open book")	Rotationally unstable, vertically stable
APC III	AP compression	Anterior and **posterior SI ligaments torn** (complete SI disruption)	Rotationally and vertically unstable; **highest blood loss and transfusion needs**
LC I	Lateral compression	Sacral buckle/impaction + oblique (horizontal) rami fractures	Stable
LC II	Lateral compression	Plus **iliac wing (crescent) fracture** or posterior SI disruption	Rotationally unstable
LC III	Lateral compression	**"Windswept pelvis"**: LC on one side, APC (open book) on the other	Unstable
VS	Vertical shear	Vertical displacement of a hemipelvis through SI joint, sacrum or ilium	Completely unstable; ipsilateral L5 transverse process avulsion
CM	Combined mechanism	Mixed patterns	

- **Lateral compression is the commonest pattern** (often elderly, side impact); **APC is associated with more bleeding**; **LC with head and**

chest injuries.

Tile (stability-based)

- **A:** stable (avulsions, iliac wing, transverse sacrococcygeal).
- **B:** rotationally unstable, vertically stable (B1 open book, B2 ipsilateral LC, B3 contralateral/bucket-handle).
- **C:** rotationally **and vertically unstable** (C1 unilateral, C2 bilateral, C3 with acetabular fracture).

Sacral fractures (Denis zones)

Zone	Location	Neurological injury
I	Lateral to the foramina (ala)	About 6%; **L5 root**
II	Through the foramina	About 28%; sciatica (unilateral roots)
III	Medial to the foramina (central canal)	About 57%; **bowel, bladder and sexual dysfunction**; includes **U/H-type (spinopelvic dissociation)** fractures

Emergency management of the bleeding pelvis

1. **ATLS**, two large-bore lines, **massive transfusion protocol** (balanced 1:1:1), **tranexamic acid within 3 hours** (CRASH-2).
2. **Pelvic binder at the level of the greater trochanters** (not the iliac crests) to reduce volume in APC injuries; internally rotate and tie the legs. Avoid over-compressing LC injuries.
3. **FAST** to look for intraperitoneal bleeding.
4. **Haemodynamically unstable despite resuscitation: preperitoneal pelvic packing** (with external fixation) and/or **angioembolisation** (arterial blush on CT; embolisation is the choice for arterial bleeding); **REBOA** in selected centres. Laparotomy if FAST positive.

5. **Anterior external fixation** (iliac crest or **supra-acetabular pins**; supra-acetabular gives better control, risk to the **lateral femoral cutaneous nerve**) or a **C-clamp** for posterior instability (risk: penetration through the greater sciatic notch).

Definitive fixation

- **Symphysis: Pfannenstiel** incision, plating of the superior surface (open book APC II/III).
- **Posterior ring: iliosacral screws** (percutaneous; S1 and S2 corridors; need good inlet/outlet/lateral imaging; danger to **L5 root** anteriorly and **S1 root** through the foramen; **sacral dysmorphism** narrows the S1 corridor), anterior SI plating, posterior tension-band plating, **lumbopelvic fixation** for spinopelvic dissociation.
- **Fragility fractures of the pelvis (FFP, Rommens and Hofmann):** elderly, low-energy LC-type injuries with sacral involvement; mobilise; fix (percutaneous screws) if pain prevents mobilisation or the fracture progresses.

Associated injuries and complications

- **Urethral injury** (males, mostly **posterior (membranous-bulbar) urethra**, APC and straddle injuries): **blood at the meatus, high-riding prostate, perineal haematoma**: **retrograde urethrogram before a catheter**; suprapubic catheter.
- **Bladder rupture** (extraperitoneal commonest; repair intraperitoneal rupture surgically).
- **Open pelvic fractures** (perineal or rectal wounds): mortality up to about 50%; **diverting colostomy** for rectal/perineal wounds.
- **Morel-Lavallee lesion:** closed degloving over the greater trochanter (shearing off the fascia); infected easily; drain/debride (percutaneous or open) before or with fixation.
- **DVT/PE** (high risk; chemoprophylaxis), **neurological deficit** (L5, S1), sexual dysfunction, chronic SI pain, malunion with limb-length discrepancy and sitting imbalance.

3. Acetabular fractures

Columns and radiology

- **Anterior column:** iliac crest (anterior part) to the pubic symphysis, including the anterior wall. **Posterior column:** greater sciatic notch to the ischial tuberosity, including the posterior wall.
- **Judet views** (45-degree obliques): **obturator oblique** (shows the **anterior column** and **posterior wall**) and **iliac oblique** (shows the **posterior column** and **anterior wall**). Six radiographic lines on AP: **iliopectineal line (anterior column)**, **ilioischial line (posterior column)**, teardrop, roof, anterior rim, posterior rim.
- **Spur sign** (obturator oblique): the intact iliac bone above the joint projecting, seen in **both-column fractures** (no articular segment attached to the axial skeleton).
- **Roof arc angles (Matta):** medial, anterior and posterior roof arcs **over 45 degrees** mean the fracture spares the weight-bearing dome (non-operative may be possible).

Letournel classification

Elementary (5)	Associated (5)
Posterior wall (**commonest overall**)	**Both column** (commonest associated; no articular fragment attached to the ilium; spur sign)
Posterior column	T-type
Anterior wall	Transverse + posterior wall (common)
Anterior column	Posterior column + posterior wall
Transverse (only elementary type through both columns)	Anterior column/wall + posterior hemitransverse

- **Elderly:** anterior column / anterior wall patterns with **quadrilateral plate** medialisation and **superomedial dome impaction ("gull**

sign") are typical (low-energy falls).

Management

- **Hip dislocation with acetabular fracture: urgent reduction** (within about 6 hours to reduce AVN), check the sciatic nerve, skeletal traction if unstable.
- **Non-operative:** undisplaced (under about 2 mm), roof arcs over 45 degrees with a congruent joint out of traction, secondary congruence of both-column fractures, small posterior wall fragments with a stable hip (**stress EUA**), elderly patients unfit for surgery.
- **Operative indications: articular displacement over 2 mm** in the dome, **unstable hip** (posterior wall over about 40-50% of the wall is usually unstable; under 20% usually stable), **incarcerated intra-articular fragments**, marginal impaction. Fix within about 2 weeks (the window closes by about 3 weeks).
- **Approaches:**

Approach	Access	Structures at risk
Kocher-Langenbeck (posterior; patient prone or lateral)	Posterior wall and column, transverse, T	**Sciatic nerve** (keep hip extended, knee flexed), **superior gluteal vessels/nerve** at the notch, **medial femoral circumflex artery** (preserve quadratus femoris and obturator externus)
Ilioinguinal (Letournel; 3 windows)	Anterior column/wall, both column	**Femoral nerve and vessels**, **lateral femoral cutaneous nerve**, **corona mortis**, spermatic cord, inguinal hernia
Modified Stoppa (anterior intrapelvic)	Quadrilateral plate, anterior column, both column	**Corona mortis**, obturator nerve and vessels

Approach	Access	Structures at risk
Extended iliofemoral	Both columns (complex late cases)	Highest **heterotopic ossification** risk

- **Elderly:** fix-and-replace (acute THA with acetabular fixation) for severe dome impaction, femoral head damage or pre-existing arthritis.

Complications

- **Post-traumatic arthritis** (commonest late complication; related to the accuracy of reduction: anatomical reduction under 1 mm gives the best results).
- **Heterotopic ossification** (posterior and extensile approaches; prophylaxis with **indomethacin** or **single-dose radiotherapy**; Brooker classification).
- **AVN of the femoral head** (with posterior dislocation), **sciatic nerve palsy** (peroneal division most affected; about 10-15%, partly iatrogenic), **DVT/PE**, infection, malreduction.

4. Hip dislocation (often with acetabular injury)

- **Posterior (about 90%):** dashboard injury, hip **flexed, adducted and internally rotated**; associated posterior wall fracture, **femoral head fracture**, **sciatic nerve injury** (peroneal division), and **ipsilateral knee injury (PCL, patella)**.
- **Anterior:** flexed, abducted and externally rotated; femoral nerve and vessels at risk.
- **Reduce urgently (within 6 hours)**; then CT for intra-articular fragments and head/wall fractures. **AVN** risk rises with delayed reduction (about 5-10% overall).
- **Thompson-Epstein** (posterior dislocation types I-V) and **Pipkin** (femoral head fractures with posterior dislocation):

Pipkin	Description	Treatment
I	Head fracture **below (caudal to) the fovea** (non-weight-bearing)	Non-operative if reduced, or excise small fragments
II	Head fracture **above (cephalad to) the fovea** (weight-bearing)	ORIF (often anterior Smith-Petersen or surgical hip dislocation)
III	I or II + **femoral neck fracture**	ORIF of neck (young) or arthroplasty (older); worst prognosis (AVN)
IV	I or II + **acetabular fracture**	Fix the acetabulum and head

Exam pearl: **APC III = most bleeding; LC = commonest; superior gluteal artery = commonest arterial bleeder; binder at the greater trochanters.** Retrograde urethrogram before catheterising a male with blood at the meatus.

Exam pearl: **Obturator oblique shows anterior column + posterior wall; iliac oblique shows posterior column + anterior wall. Spur sign = both-column fracture.** Posterior wall = commonest acetabular fracture; Kocher-Langenbeck endangers the sciatic nerve and MFCA.

Tibial plateau and pilon

1. Tibial plateau fractures

Anatomy and mechanism

- The **lateral plateau** is **convex** and higher; the **medial plateau** is **concave**, larger and stronger (more load).
- Mechanism: **valgus load with axial force** (lateral plateau, commonest), varus (medial plateau, higher energy, often **fracture-dislocation**).
- **Lateral plateau fractures are commonest (about 55-70%).**
- Associated injuries: **lateral meniscus tears** (commonest with lateral plateau; often peripheral and incarcerated), **medial meniscus**, **MCL and ACL injuries**, **peroneal nerve** (lateral fracture-dislocation), **popliteal artery** (medial plateau and Schatzker IV/VI, bicondylar fractures; **ankle-brachial index under 0.9 = arterial study**), **compartment syndrome** (especially Schatzker VI).

Schatzker classification

Type	Description	Typical patient / mechanism	Treatment principle
I	Lateral **split** (wedge)	Young, dense bone	Percutaneous screws or buttress plate; check for meniscal entrapment
II	Lateral **split-depression**	**Commonest**	Elevate the depressed fragment, **subchondral bone graft/substitute**, raft screws, lateral buttress plate

Type	Description	Typical patient / mechanism	Treatment principle
III	Lateral **pure depression**	**Osteoporotic** elderly	Elevation through a cortical window, graft, raft screws (IIIa lateral, IIIb central)
IV	**Medial plateau**	High energy; fracture-dislocation (Moore); **popliteal artery and peroneal nerve at risk**	Posteromedial approach, buttress plate; poor prognosis
V	**Bicondylar** with metaphysis intact	High energy	Dual plating / lateral locking ± medial plate
VI	Bicondylar with **metaphyseal-diaphyseal dissociation**	Highest energy; **compartment syndrome**	**Staged**: spanning external fixator, then ORIF (dual incisions) when soft tissues allow; or hybrid/circular frame

- **Three-column concept (Luo; CT-based):** lateral, medial and **posterior** columns (the posterior column, especially **posteromedial fragments**, is missed by Schatzker; needs a **posteromedial (Lobenhoffer) or posterior approach** and buttress plating).
- **Moore classification** (fracture-dislocations): split, entire condyle, rim avulsion, rim compression, four-part.

Investigation

- AP, lateral, oblique; **CT** for every operative plateau; **MRI** shows meniscal/ligament injuries (commonly present).
- **Assess soft tissues** (fracture blisters, swelling: **"wrinkle sign"** shows readiness for surgery, usually 7-14 days).

Management

- **Non-operative:** undisplaced or minimally displaced, stable to varus/valgus stress in full extension (under about 10 degrees), elderly low-demand.
- **Operative indications: articular step-off over 2-3 mm** (debated: stability and alignment matter more), **condylar widening over 5 mm**, **instability (over 10 degrees varus/valgus in extension)**, **medial plateau fractures** (inherently unstable), open fractures, compartment syndrome, vascular injury.
- **Principles:** restore the **mechanical axis and joint stability** first (more important than perfect articular congruity), elevate the joint surface, **support it with subchondral rafting screws and bone graft/substitute** (calcium phosphate cement resists subsidence best), **buttress** the metaphysis, repair menisci.
- **Approaches:** anterolateral (lateral plateau; submeniscal arthrotomy to see the joint); posteromedial (Lobenhoffer, between the medial gastrocnemius and pes anserinus/semimembranosus); posterior (Burks-Schaffer, Carlson); avoid a single midline incision with extensive stripping for bicondylar fractures (**dual incisions with a skin bridge over about 7 cm** historically; soft tissue is the priority).
- **Staged protocol for high-energy fractures: spanning external fixation** (ligamentotaxis), CT in the frame, definitive fixation once the soft tissues settle.
- **Arthroscopy-assisted reduction (ARIF)** for lateral split/depression fractures.
- **Elderly with severe comminution/arthritis:** acute TKA in selected cases.

Complications

- **Infection and wound breakdown** (bicondylar, early surgery through poor soft tissue), **post-traumatic arthritis** (malalignment and instability more than step-off), **stiffness/arthrofibrosis**, loss of

reduction (subsidence), **compartment syndrome**, **peroneal nerve injury**, DVT, malunion.

2. Tibial shaft fractures (brief link)

- Commonest long-bone fracture; **highest rate of open fractures** and compartment syndrome.
- **Reamed intramedullary nail** for most displaced fractures (**SPRINT**: no difference between reamed and unreamed overall; possible benefit of reaming in closed fractures). **Suprapatellar (semi-extended) nailing** eases proximal and distal third fractures (less anterior knee pain debated).
- **Proximal third fractures** tend to **valgus and procurvatum (apex anterior)** with nailing: use **blocking (Poller) screws**, a more lateral/proximal start point, extended positioning, plates.
- **Distal third**: nail with blocking screws, or plate; fix the fibula if needed for alignment.
- Non-operative (Sarmiento functional bracing) for low-energy, stable fractures with acceptable alignment (under 5 degrees varus/valgus, under 10 degrees AP angulation, under 1 cm shortening, under 10 degrees rotation).
- **Anterior knee pain** is the commonest complication after tibial nailing.

3. Tibial pilon (plafond) fractures

Mechanism and features

- **Axial loading**: the talus is driven into the distal tibial articular surface (falls from height, road traffic accidents). Distinct from rotational ankle fractures.
- **Soft-tissue injury is the dominant problem** (thin anterior and medial soft tissue envelope).

- Fragments (Chaput's): typically **medial**, **anterolateral (Chaput, attached to AITFL)**, **posterolateral (Volkmann, attached to PITFL)**, with central die-punch impaction; associated fibula fracture in about 75%.
- **Associated injuries:** spine fractures, calcaneus fractures, contralateral injuries (falls from height).

Classification

Ruedi-Allgower	Description
I	Undisplaced cleavage fracture of the joint
II	Displaced articular fracture, minimal comminution
III	Comminuted and impacted articular surface

- **AO/OTA 43:** A extra-articular, B partial articular, C complete articular (C3 = comminuted articular + metaphyseal).
- **Tscherne** for closed soft-tissue injury (C0-C3).

Management: the staged protocol

1. **Stage 1 (urgent): spanning external fixator** (tibia to calcaneus/first metatarsal) to restore length, alignment and rotation by ligamentotaxis; ± **fibular fixation** (if the plafond team and soft tissue plan allow; malreduced fibula makes later tibial reduction impossible).
2. **CT in the frame**: plan approaches.
3. **Stage 2 (when soft tissues allow, usually 7-21 days; "wrinkle sign", blisters re-epithelialised): ORIF** of the articular surface with plates (anterolateral, anteromedial, posterolateral, medial approaches depending on fragments), bone graft for metaphyseal defects.
4. **Rüedi-Allgöwer principles:** (1) restore fibular length, (2) reconstruct the articular surface, (3) bone graft the metaphyseal defect, (4)

buttress the medial tibia.

5. **Alternatives:** definitive **circular (Ilizarov) or hybrid external fixation** with limited internal fixation when soft tissues are poor; **primary ankle arthrodesis** for unreconstructable fractures.
6. **Approaches: anterolateral** (between the EDL/peroneus tertius and the fibula; superficial peroneal nerve at risk), **anteromedial** (medial to tibialis anterior; thinnest soft tissue), **posterolateral** (between peroneals and FHL; sural nerve), **medial**. Skin bridges of at least 5-7 cm between incisions (debated).

Complications

- **Wound dehiscence and deep infection** (historically up to 30-40% with early ORIF; staged protocols reduced this to under 5-10%).
- **Post-traumatic ankle arthritis** (common; related to the initial cartilage damage more than reduction quality), malunion (varus), non-union (metaphyseal), stiffness, chronic pain, complex regional pain syndrome.

> Exam pearl: **Schatzker II (split-depression) is the commonest; IV (medial) = fracture-dislocation with popliteal artery and peroneal nerve risk; VI = metaphyseal-diaphyseal dissociation with compartment syndrome.** The **lateral meniscus** is the commonest associated soft-tissue injury.

> Exam pearl: **Pilon = axial load; soft tissue dictates timing; stage it: span, scan, plan, then fix (7-21 days).** Rüedi-Allgöwer's four steps start with **fibular length**.

Ankle fractures

1. Applied anatomy

- The ankle mortise: tibial plafond, medial malleolus, lateral malleolus (fibula, the main lateral restraint). The **talus is wider anteriorly** (stable in dorsiflexion).
- **Ramsey and Hamilton: 1 mm of lateral talar shift reduces tibiotalar contact area by about 42%** (the basis for anatomical reduction).
- **Medial ligament (deltoid): superficial** (crosses ankle and subtalar; tibionavicular, tibiospring, tibiocalcaneal) and **deep** (deep anterior and posterior tibiotalar; **the primary restraint to lateral talar shift**).
- **Lateral ligaments: ATFL** (weakest, commonest sprain; tight in plantarflexion), **CFL** (crosses ankle and subtalar joints), PTFL (strongest).
- **Syndesmosis: AITFL**, **PITFL** (strongest component, with its deep part the inferior transverse ligament), interosseous ligament, interosseous membrane.
- The posterior malleolus is attached to the PITFL; a posterior malleolar fracture can destabilise the syndesmosis.

2. Classifications

Lauge-Hansen (mechanism: position of the foot + direction of force)

Type	Frequency	Sequence of injury	Fibular fracture
Supination-external rotation (SER)	**Commonest (about 40-75%)**	I AITFL, II **short oblique/spiral fibula at the level of the syndesmosis** (anteroinferior to posterosuperior), III PITFL or posterior malleolus, IV medial malleolus fracture or deltoid rupture	Weber B
Supination-adduction (SAD)	About 10-20%	I transverse fibular avulsion below the joint (or lateral ligament rupture), II **vertical medial malleolus fracture** with **medial plafond impaction** (fix and elevate)	Weber A
Pronation-external rotation (PER)	About 7-20%	I medial malleolus/deltoid, II AITFL, III **high spiral fibula fracture above the syndesmosis**, IV PITFL/posterior malleolus	Weber C (includes **Maisonneuve**)
Pronation-abduction (PAB)	About 5-20%	I medial malleolus/deltoid, II syndesmosis/posterior malleolus, III **comminuted/transverse fibula fracture at or above the syndesmosis** (lateral comminution)	Weber B/C

- Remember: the **first word is the foot position**, the second the force; the **first injured structure is on the tension side** (supination = lateral first; pronation = medial first).

Danis-Weber (level of the fibular fracture relative to the syndesmosis)

- **A:** below the syndesmosis (infra-syndesmotic); syndesmosis intact.
- **B:** at the level of the syndesmosis (trans-syndesmotic); syndesmosis may be injured.
- **C:** above the syndesmosis; syndesmosis torn (needs assessment and usually fixation).

Special patterns

- **Maisonneuve fracture: proximal fibula fracture + syndesmotic and interosseous membrane disruption + medial injury** (PER). Always palpate the whole fibula in an "isolated" medial malleolar fracture or deltoid injury. Treat with **syndesmotic fixation** (fibula itself usually not fixed).
- **Bosworth fracture-dislocation:** fibula **trapped behind the posterolateral tibia (incisura)**; irreducible by closed means: **emergency open reduction**.
- **Tillaux-Chaput** (AITFL avulsion from the tibia) and **Wagstaffe-Le Fort** (AITFL avulsion from the fibula) fragments.
- **Posterior malleolus** fractures: **Haraguchi classification** (CT): I posterolateral oblique (commonest), II medial extension, III small shell (rim). **Bartoníček-Rammelt** classification relates to the incisura.

3. Assessment

- **Ottawa ankle rules** (radiograph if pain in the malleolar zone plus tenderness at the posterior edge/tip of either malleolus, or inability to bear weight four steps immediately and in the department; foot series if tenderness at the base of the fifth metatarsal or navicular).
- **Radiographs:** AP, lateral, **mortise view** (15-20 degrees internal rotation).

  - **Medial clear space: over 4-5 mm** (or greater than the superior clear space) = **deltoid injury / lateral talar shift**.
  - **Tibiofibular clear space** (AP, 1 cm above the plafond): **over 5-6 mm = syndesmotic injury** (most reliable radiographic measure).
  - **Tibiofibular overlap:** under 6 mm on AP or under 1 mm on mortise = syndesmotic injury.
  - **Talocrural angle** (about 83 degrees ±4): assesses fibular length; **dime sign** (continuous curve between the fibular tip and lateral talus) shows fibular length.
- **Stress views: gravity or manual external rotation stress** (medial clear space over 5 mm) or **weight-bearing radiographs** decide whether an isolated Weber B (SER) fracture is stable: **a weight-bearing mortise with a normal medial clear space indicates stability** even if the gravity stress view widens.
- **CT** for posterior malleolar fractures, pilon-type impaction, and syndesmosis assessment; **MRI** for occult injuries.

4. Management

Stable injuries (non-operative)

- **Weber A**, **isolated undisplaced Weber B with a stable mortise** (normal medial clear space on weight-bearing film), isolated undisplaced medial malleolar fractures.
- Treatment: **walking boot or cast with early weight bearing as tolerated** for 6 weeks. The **AIM trial** (elderly, unstable ankle fractures, close-contact casting vs ORIF) showed **equivalent function at 6 months** with skilled casting; **CROSSBAT** (Kortekangas): undisplaced/minimally displaced Weber B can be treated non-operatively (casting for 3 weeks equals 6 weeks).

Unstable injuries (operative)

- **Bimalleolar**, **trimalleolar**, unstable SER IV equivalents (fibula + deltoid with talar shift), syndesmotic injuries, **open fractures**,

irreducible dislocations, **talar shift**.

- **Emergency: reduce a dislocated ankle immediately** (skin and neurovascular compromise), splint, elevate; ORIF when swelling allows (within about 24 hours or after 7-10 days).
- **Fibula:** restore **length and rotation**: lag screw with **one-third tubular neutralisation plate (lateral)** or **posterolateral antiglide plate** (Weber B; biomechanically stronger, less hardware prominence, risk to peroneal tendons distally). Intramedullary fibular nail in poor soft tissue/elderly.
- **Medial malleolus: two 4.0 mm partially threaded cancellous screws** perpendicular to the fracture, or tension-band wiring for small fragments; **vertical (SAD) fractures need horizontal screws or an antiglide plate**, plus elevation of medial plafond impaction. The **deltoid ligament usually heals without repair** once the fibula is reduced (repair if the medial clear space does not reduce: interposed deltoid or tibialis posterior tendon; some advocate repair in high-demand athletes).
- **Posterior malleolus:** fix when **over about 25-33% of the articular surface** (traditional rule), when there is **posterior talar subluxation**, an **intercalary/impacted fragment**, or when it destabilises the syndesmosis. Current evidence favours **direct fixation (posterolateral approach) of larger fragments** because it restores syndesmotic stability and incisura congruity better than a syndesmotic screw.
- **Syndesmosis:**
  - **Intraoperative tests: Cotton (hook) test** (lateral pull on the fibula with a bone hook: over about 2 mm widening), external rotation stress.
  - **Fixation: screw(s) (3.5 or 4.5 mm, through 3 or 4 cortices, 2-5 cm above the joint, parallel to the plafond, angled about 25-30 degrees anteriorly from posterior)** or **suture-button (dynamic) fixation**. **Suture-buttons** give fewer malreductions and less need

for removal (better functional scores in some trials); screws may be removed after about 3 months (debated).
    - **Accurate reduction is critical**: **direct visualisation of the incisura** or intraoperative CT, because **malreduction (up to about 30-50% on CT)** is the main cause of poor outcome. Clamp along the transmalleolar axis to avoid over-compression or malrotation.
- **Post-operative:** early range of motion; weight bearing in a boot from about 2-6 weeks (early weight bearing is safe in most fixed fractures).

Special groups

- **Diabetic patients (especially with neuropathy):** high complication rates (infection, Charcot, loss of fixation). **"Super-construct" fixation** (longer plates, multiple syndesmotic screws, transarticular pins), prolonged non-weight bearing (about double the usual time).
- **Elderly osteoporotic:** locking plates, fibular nails, **tibiotalocalcaneal (hindfoot) nail** for very poor bone/soft tissue; skilled close-contact casting (AIM).
- **Open fractures:** debridement, antibiotics, stabilisation (external fixation or internal fixation depending on contamination), soft-tissue cover.

5. Ankle sprains and related injuries

- **Lateral ankle sprain:** ATFL then CFL. **Anterior drawer** (ATFL), **talar tilt** (CFL). **Functional treatment** (brace, early motion, proprioceptive rehabilitation) for all grades acutely.
- **Chronic lateral ankle instability** after failed rehabilitation: **anatomical repair (Broström-Gould: ATFL/CFL repair reinforced with the inferior extensor retinaculum)**; anatomical reconstruction with tendon graft for poor tissue, failed repair,

generalised laxity or high BMI; non-anatomical (Chrisman-Snook, Watson-Jones, Evans) rarely now (stiffness of the subtalar joint).

- **High ankle sprain (syndesmosis): squeeze test**, external rotation (Kleiger) test; longer recovery; stabilise if unstable on stress/arthroscopy.
- **Missed injuries around a "sprain": lateral process of the talus fracture** (snowboarder's), **anterior process of the calcaneus fracture**, **base of fifth metatarsal fracture**, **osteochondral lesion of the talus**, **peroneal tendon subluxation/tear**, **Maisonneuve fracture**.
- **Osteochondral lesions of the talus: anterolateral** (shallow, wafer-shaped, traumatic) vs **posteromedial** (deeper, cup-shaped, often atraumatic). **Berndt-Harty** staging; MRI/CT; **arthroscopic debridement and microfracture/bone-marrow stimulation for lesions under about 1.5 cm^2 (or under 15 mm diameter)**; osteochondral autograft (OATS) or other cartilage procedures for larger lesions.

6. Complications of ankle fractures

- **Post-traumatic arthritis** (malreduction of fibular length/rotation, syndesmotic malreduction, articular damage).
- **Wound complications and infection** (smokers, diabetes, early surgery through swollen skin).
- **Syndesmotic malreduction**, **broken syndesmotic screws** (usually harmless), loss of reduction.
- **Superficial peroneal nerve** injury (lateral incision, about 7 cm above the tip of the fibula it pierces the fascia), sural nerve (posterolateral approach), saphenous nerve (medial).
- Complex regional pain syndrome, stiffness, DVT.

Exam pearl: **SER is the commonest Lauge-Hansen type (fibula at the syndesmosis, anteroinferior to posterosuperior). PER IV gives a high fibular fracture; Maisonneuve = proximal fibula + syndesmotic disruption. 1 mm talar shift = 42% loss of contact area.**

Exam pearl: **Medial clear space over 4-5 mm = deltoid injury; tibiofibular clear space over 5-6 mm = syndesmotic injury.** Weight-bearing radiographs decide stability of an isolated Weber B. **Syndesmotic malreduction is the main cause of poor outcome; visualise the incisura.**

Proximal humerus and shoulder girdle

1. Proximal humerus fractures

Anatomy

- Four parts (**Codman**): **head** (articular segment), **greater tuberosity** (supraspinatus, infraspinatus, teres minor), **lesser tuberosity** (subscapularis), **shaft**.
- **Head blood supply:** the **posterior humeral circumflex artery** supplies most of the head (Hettrich, 2010; about 64%); the **anterior humeral circumflex artery** and its **arcuate artery (of Laing)**, entering at the bicipital groove, was classically considered the main supply.
- **Hertel's predictors of head ischaemia: metaphyseal extension under 8 mm** attached to the head (posteromedial calcar), **disrupted medial hinge (over 2 mm displacement)**, and the **anatomical neck fracture** pattern. Head ischaemia does not always mean AVN.
- **Deforming forces:** greater tuberosity pulled **superiorly and posteriorly** (supraspinatus/infraspinatus); lesser tuberosity **medially** (subscapularis); shaft **medially and anteriorly** (pectoralis major).
- **Axillary nerve** runs about **5-7 cm below the acromion** (deltoid-splitting approach danger zone); the **arcuate artery** and **cephalic vein** in the deltopectoral approach.

Neer classification

- A "part" is displaced if **over 1 cm displacement or over 45 degrees angulation** (for the greater tuberosity, **over 5 mm displacement** is significant in young/overhead patients, as superior displacement causes impingement).
- One-part (undisplaced, about 80%), two-part (surgical neck commonest two-part; greater tuberosity; anatomical neck rare), three-part (surgical neck + one tuberosity), four-part (both

tuberosities + surgical neck), **fracture-dislocations** and **head-splitting** fractures.

- **Valgus-impacted four-part fracture:** better prognosis (medial periosteal hinge preserved, lower AVN).
- **AO/OTA 11** classification also used.

Management

- **Non-operative** (sling, early pendulum exercises at about 2 weeks): most fractures, especially **one-part** and in the **elderly**. The **ProFHER trial** (2015, elderly displaced surgical-neck-involving fractures): **no benefit of surgery over non-operative care** at 2 and 5 years.
- **Isolated greater tuberosity fracture: fix if displaced over 5 mm** (screws, suture-bridge/tension band) in active patients.
- **Two/three-part fractures in active patients: proximal humeral locking plate** (deltopectoral or deltoid-split approach). **Key technical points:** restore the **medial calcar support** (an inferomedial "calcar screw"; medial comminution predicts varus collapse), avoid **plate placed too high** (subacromial impingement; plate about 8 mm below the top of the greater tuberosity), **avoid screw cut-out** (commonest complication: primary or secondary intra-articular screw penetration with varus collapse), use **tuberosity sutures** through the cuff. Fibular strut allograft in the canal for medial support.
- **Intramedullary nails** for two-part surgical neck fractures (straight nails reduce cuff damage).
- **Four-part fractures, head-split, fracture-dislocations in the elderly:**
  - **Reverse total shoulder arthroplasty (RTSA)** is now preferred in elderly patients (over about 70): **more predictable function**, does not depend on tuberosity healing (though tuberosity repair improves rotation).

  - **Hemiarthroplasty** in younger patients where tuberosity healing is likely: outcome **depends on tuberosity healing** (correct height and version: top of the head about 5-8 mm above the greater tuberosity, retroversion about 20-30 degrees).
  - **ORIF** in young patients (attempt to preserve the head; AVN may still occur).
- **Complications: AVN** (four-part and head-split, about 10-30%+), malunion (varus, tuberosity), non-union (surgical neck), **screw cut-out**, stiffness, **axillary nerve injury**, infection.

2. Shoulder dislocation (acute, trauma aspect)

- **Anterior (about 95-97%):** abduction-external rotation; associated **Bankart lesion** (anteroinferior labrum/capsule), **Hill-Sachs** (posterosuperolateral head impaction), **greater tuberosity fracture** (in older patients; often reduces with the head), **axillary nerve injury** (commonest nerve), **rotator cuff tear** in patients over 40 (**test cuff strength after reduction; MRI/ultrasound if weak**).
- **Recurrence after first dislocation** is inversely related to age: **up to about 90% under 20 years**, much lower over 40. Arthroscopic Bankart repair after first dislocation lowers recurrence in young athletes.
- **Posterior: seizures, electrocution**, **lightbulb sign**, **trough (reverse Hill-Sachs/McLaughlin) sign**, inability to externally rotate; **often missed** (always get an **axillary lateral** view). Reverse Hill-Sachs over about 25% of the head: McLaughlin procedure (subscapularis/lesser tuberosity transfer); over about 50%: arthroplasty/allograft.
- **Luxatio erecta** (inferior): arm locked in abduction; neurovascular injury common.

3. Clavicle fractures

- Commonest fracture of the shoulder girdle; **middle third (about 80%)** (Allman group I), lateral third (about 15%, group II), medial

third (about 5%, group III).

- **Midshaft:** most heal non-operatively. **Displaced/shortened midshaft fractures** (100% displacement, **shortening over about 2 cm**, comminution) have higher **non-union (about 15%)** and **symptomatic malunion** rates without surgery (**COTS trial**, 2007: **plate fixation lowered non-union and improved function** in displaced fractures; later trials show smaller differences). **Absolute indications: open fracture, skin tenting with impending perforation, neurovascular injury**, floating shoulder (relative), polytrauma.
- **Fixation:** superior or anteroinferior plate (anteroinferior: less prominence, longer screws, less subclavian vessel risk) or intramedullary devices.
- **Lateral third (Neer): Type I** (lateral to the coracoclavicular ligaments, stable), **Type II** (medial to CC ligaments; the medial fragment detaches from the CC ligaments, **high non-union (about 30%)**: **fix** with hook plate, lateral locking plate or CC suspension), **Type III** (intra-articular into AC joint; late AC arthritis), IV (children, periosteal sleeve), V (comminuted).
- **Medial clavicle / sternoclavicular:** posterior SC dislocation endangers great vessels and trachea (**CT; reduce in theatre with a thoracic surgeon available**); medial physis closes at about 23-25 years (physeal injuries mimic dislocations).

4. Acromioclavicular (AC) joint injuries

- **Rockwood classification:**

Type	AC ligament	CC ligament	Displacement	Treatment
I	Sprained	Intact	None	Sling
II	Torn	Sprained	Up to 25% (subluxed)	Sling

Type	AC ligament	CC ligament	Displacement	Treatment
III	Torn	Torn	**25-100%** CC distance increase	**Non-operative first** in most; surgery for heavy labourers/overhead athletes or failed rehab
IV	Torn	Torn	**Clavicle posterior** through trapezius (axillary view)	Surgery
V	Torn	Torn	**100-300%** (deltotrapezial fascia stripped)	Surgery
VI	Torn	Torn	Clavicle **inferior** (subacromial or subcoracoid)	Surgery (rare)

- **Zanca view** (10-15 degrees cephalic tilt) for the AC joint; **normal CC distance about 11-13 mm**.
- Surgery: anatomical CC ligament reconstruction (graft), suture-button suspension, hook plate (acute), **Weaver-Dunn** (coracoacromial ligament transfer, historical).

5. Scapula fractures

- High-energy injuries; **associated injuries in about 80-90%** (rib fractures, pneumothorax, pulmonary contusion, head and spine injury).
- **Body fractures** (commonest): mostly non-operative. Operate for significant **medialisation of the glenoid (over 20 mm)**, **angulation over 45 degrees**, **glenopolar angle under 22 degrees**.
- **Glenoid fractures (Ideberg):** fix if **articular step-off over 4-5 mm**, or rim fractures causing **instability (over about 20-25% of the**

glenoid).

- **Floating shoulder:** ipsilateral clavicle and glenoid neck fractures (superior shoulder suspensory complex, **Goss**: double disruption of the SSSC). Fix one or both when displaced.
- **Scapulothoracic dissociation:** lateral displacement of the scapula (**scapular index**), often with **brachial plexus avulsion and subclavian/axillary artery injury**; high mortality; flail arm, often amputation.

6. Humeral shaft fractures

- **Holstein-Lewis fracture:** spiral fracture of the distal third with **radial nerve** entrapment risk.
- **Non-operative (Sarmiento functional brace)** is standard: acceptable alignment **under 20 degrees anterior angulation, under 30 degrees varus, under 3 cm shortening**. **FISH trial** (2021) and others: surgery gives faster early recovery but similar long-term function; non-union about 10-15% with bracing (proximal third and transverse patterns more).
- **Operative indications: open fracture**, **vascular injury**, **floating elbow**, polytrauma (to allow crutch use), **pathological fracture**, bilateral fractures, failed bracing, segmental fractures, intra-articular extension, brachial plexus injury.
- **Plating** (anterolateral, posterior or MIPO) is preferred over **antegrade nailing** (nailing causes more **shoulder pain** and reoperation; useful in pathological fractures).
- **Radial nerve palsy with a closed fracture** (about 10-18%): mostly **neurapraxia**: **observe** (about 70-90% recover spontaneously; wait about 3-4 months, then EMG; explore if no recovery). **Explore early** for **open fractures** (nerve may be lacerated) and palsy with an **open or high-energy fracture needing fixation**. **Palsy after closed manipulation** is debated (traditionally explore). Tendon transfers

(pronator teres to ECRB, FCR/FCU to EDC, palmaris longus to EPL) if no recovery.

Exam pearl: **Neer part = over 1 cm or 45 degrees (greater tuberosity over 5 mm). ProFHER: elderly displaced fractures do as well without surgery. Reverse arthroplasty is preferred for four-part fractures in the elderly; hemiarthroplasty depends on tuberosity healing. Medial calcar support prevents varus collapse after locking plates.**

Exam pearl: **Posterior shoulder dislocation (seizure, electrocution) = lightbulb sign, missed without an axillary view. Neer II lateral clavicle fracture has a high non-union rate: fix. Rockwood III AC injury: non-operative first. Radial nerve palsy with closed humeral shaft fracture: observe.**